

AI GI Care - Primary Care Visit Note

SAMPLE DOCUMENT - test data only, not a real patient record

Patient: Jordan A. Ellis DOB: 1978-04-12 (47y) Sex: Male
MRN: SAMPLE-104829 Visit date: 2026-08-04
Provider: Dr. Maria Chen, MD - Riverside Family Medicine

Chief complaint

Intermittent upper abdominal pain and reflux for approximately 4 months, worse after meals and at night.

History of present illness

Burning epigastric discomfort 3-4 times per week, rated 5/10, radiating to the chest. Partial relief with over-the-counter antacids. Reports early satiety and 3 kg unintentional weight loss over 3 months. No melena, no hematemesis, no dysphagia. No NSAID use.

Past medical history

Hypertension (2019). Hyperlipidemia (2021). GERD (2024).

Medications

Lisinopril 10 mg daily. Atorvastatin 20 mg nightly.
Omeprazole 20 mg daily (started 2026-06-15).

Allergies

Penicillin - rash.

Examination

BP 128/78, HR 74, Temp 36.8 C, BMI 27.4. Abdomen soft, mild epigastric tenderness, no guarding, no palpable mass. Bowel sounds normal.

Assessment and plan

- Persistent dyspepsia with alarm features (weight loss, age > 45).
- Referred to gastroenterology for evaluation and consideration of upper endoscopy.
- Continue omeprazole 20 mg daily pending GI review.
- H. pylori stool antigen and CBC/CMP ordered - see lab report.